

accommodated in the corridors, a space called by participants "stretchers" (DOC1, DOC2).

Care management

In the context of the work process of nurses at EHS, nursing care management involves activities performed by nurses to provide care, provide support for planning, performing and organizing work, articulating care and management in their work practice. The activities that promote this articulation involve human resources management, assistance delivery, decision-making and provision of material and infrastructure resources, which were named by participants through the expression "put out fire".

When we take the duty, it is basically putting out the fire, this is missing, that is missing, the medication has to be taken from the pharmacy, the HU (extra medication request), there is the patient who has to go there to see, the other is without the medicine, the other is without the prescription, has exam to take, was missing it. So, we arrive and try to organize the remaining issues first, so you can start to see, assess the patients, do your things, make bandages, see the patients intubated. (I15)

You have a huge variety of patient cases, so you will put out a fire, because one day there is an employee missing, then you get ready to give care, then there is a stop, then there are more patients, then you have to transfer patients, then you have to be organizing, assessing who is more serious or less serious and everything else too, you do not have a work process already defined, I call it fire extinguishing, you are there to meet the most urgent causes. (FG1)

Thus, the work process is effective through the constant articulation between the management and the care dimensions, merging and materializing in care management, focusing on the immediate resolution of patients' needs or unit problems. The term "put out fire" allows us to infer decision-making as a process of prioritization of tasks, in a sector contextualized by the unpredictability, the immediacy and intensity of care needs and interruptions in care delivery. Characteristics abstracted by sector demand, concretized in the speeches due to complications, in which the dichotomy continuous care versus emergency care stands out.

There is no time, it is a kind of extinguishing fire, it is just doing "have a probe in that", "aspire that", "bandage that", "pass that one here", "look for that one". (FG2)

Sometimes you program yourself so you leave care activities already listed, but sometimes you can't follow that because there are so many complications that happen. (I12)

Sometimes you are in a rush, in a call, in an emergency, in a stop, but at the same time there is someone calling you, it is familiar asking you something. (I11)

Care is interspersed with the different management and care priorities, emerging "care in parentheses" exemplified in the following statement.

Yesterday I had to see the NO (Nothing Orally) There were some surgeries that were canceled and the patients were in NO, when I was

talking to the secretary the technique came to pass me that I had a patient who was very, very hypertensive he was heart disease and had no prescribed medication, ok what I did? I wrote a note to the secretary saying everything I wanted and went to see the patient, saw the patient talked to the doctor, and went back to the secretary to see the operating room staff. So, I go (pause) it's like parentheses are created within a service you know? When you have a speech that you are writing and you want to (pause) create a parenthesis, it's an action in parentheses! If I were to give a concrete example. (I7)

Thus, the term "put out fire" reveals intermittent caution in the face of multiple work process tasks. This makes nurses use decision-making as a strategy to manage care in the face of the several concurrent and/or sequential tasks that arise from the various simultaneous priorities present in the EHS.

DISCUSSION

The participants' profile consisted of experienced professionals, contrasting with their short time working in the sector, partly explained by the holding of a large competition about three years before the research was conducted. The profile showed a slight predominance of females, with high level of education. The profiles of emergency service nurses generally differ from other settings, with the male population having more experience and advanced qualifications predominating. However, this variation has a cultural influence, so that in underdeveloped countries there is a predominance of women and with less qualifications⁽⁹⁾.

The environment of EHS is marked by overcrowding and patient permanence even after the need for acute care ceases, which generates a large number of patients with continuous care demands with different levels of complexity⁽¹⁰⁾. The permanence of inpatients is a reality in the internationally described EHS that increases the nursing workload, compromising the quality of nursing care and patient safety⁽¹¹⁻¹⁵⁾.

Inpatient care compromises care for patients with emergency demands, since tasks related to continuous care require more time from nurses. A study conducted at a trauma care center showed that the greater the number of patients under the care of a single nurse, the longer the time for assessment and diagnosis, influencing emergency service performance and patient outcomes⁽¹⁶⁾. Similarly, nurses reported difficulties in providing care to patients with prolonged stay in Swedish hospital emergencies, as it is a service that is not designed, equipped or structured for this purpose⁽¹²⁾.

The clash between continuous care and emergency care leads nurses to face a loss of identity. Emergency nurses expect to serve patients in life-limiting demands. However, as your work process is taken care of by continuous care, you lose the clarity of roles and work identity and, consequently, control of your environment^(13,17).

The service receives a large number of patients requiring intensive care, which is consistent with the level of complexity of the EHS. On the other hand, it is known that the inappropriate use of high complexity services is seen as a frustration⁽¹⁰⁾ and a source of overload for the team, which may make it difficult to treat the most severe patients⁽¹⁸⁾. In this sense, the emergency service environment shapes the work process of nurses, in which the high and differentiated demand, which involves emergency care and continuous care, implies multiple tasks management,